

PRODUCT LITERATURE

PROMETIL TABLET 5 MG

Each tablet contains

Prochlorperazine Maleate 5 mg

Preservatives in Syrup

Sodium Benzoate 0.1% w/w

Product Description

Odourless, Round, white tablet with J51 embossed with score on one side and TPI embossed on other side.

Pharmacodynamics properties

Phenothiazines with piperazine structure Prochlorperazine maleate is a phenothiazine. Prochlorperazine has a wide range of activity arising from its depressant actions on the CNS and its alpha-adrenergic blocking and weaker antimuscarinic properties. It inhibits dopamine and prolactin-release-inhibitory factor, thus stimulating the release of prolactin. The turnover of dopamine in the brain is increased. There is evidence that the antagonism of central dopaminergic function is related to the therapeutic effect in psychotic conditions. Prochlorperazine has sedative properties but tolerance to the sedation usually develops rapidly. Prochlorperazine has anti-emetic, anti-pruritic, serotoninblocking, and weak antihistamine properties and slight ganglion-blocking activity. It inhibits the heat regulating centre, can relax smooth muscle and has membrane stabilising and hence local anaesthetic properties. Its actions on the autonomic system produce vasodilatation, hypotension and tachycardia. Salivary and gastric secretions are reduced.

Pharmacokinetic properties

Prochlorperazine is well absorbed from the GI tract but is subject to considerable first pass metabolism from the gut wall. It is also extensively metabolised in the liver and is excreted in the urine and bile. Plasma concentrations following oral administration are much lower than those following intramuscular injection, and are subject to wide inter-subject variation. There is no simple correlation between plasma concentrations of prochlorperazine and its metabolites, and therapeutic effect. Prochlorperazine may be metabolised by hydroxylation and conjugation with glucuronic acid, N-oxidation, oxidation of the sulfur atom and dealkylation. Plasma half-life is reported to be only a few hours but elimination of the metabolites may be very prolonged. Prochlorperazine is extensively bound to plasma proteins, widely distributed in the body (it crosses the blood/brain barrier) and its metabolites cross the placental barrier and are excreted in milk. The rate of metabolism and excretion decreases in old age.

Indication

For the management of nausea and vomiting.

Recommended Dose

Adult: 1 - 2 tablets, 3 or 4 times a day

Route of Administration

Oral

Contraindications

Known Hypersensitivity to the active substance or to any of the excipients.

Warning and Precaution:

Cross-sensitivity and/or related problems
Patients sensitive to one phenothiazine may be sensitive to other phenothiazines also.

Carcinogenicity/Mutagenicity

Long-term animal studies to evaluate carcinogenic or mutagenic potential have not been performed.

Tumorigenicity

Phenothiazines produce an elevation in prolactin concentrations, which persists throughout administration. Tissue culture experiments indicate that approximately one third of human breast cancers are prolactin-dependent in vitro, a factor of potential importance if the prescription of these medications is contemplated in a patient with a previously detected breast cancer. Although disturbances such as galactorrhea, amenorrhea, gynecomastia, and impotence have been reported, the clinical significance of elevated serum prolactin concentrations is unknown for most patients. An increase in mammary neoplasms has been found in rodents after chronic administration of antipsychotic medications. However, neither clinical studies nor epidemiologic studies conducted to date have shown an association between long-term administration of these medications and mammary tumorigenesis; the available evidence is considered too limited to be conclusive at this time.

Pediatrics

Children are prone to develop neuromuscular or extrapyramidal reactions, especially dystonias, and should be closely monitored while receiving therapeutic doses of Phenothiazines. Children with acute illnesses, such as chickenpox, central nervous system (CNS) infections, measles, gastroenteritis, and dehydration, are especially at risk.

Geriatrics

Geriatric patients tend to develop higher plasma concentrations of Phenothiazines. Therefore, these patients usually require lower initial dosage and a more gradual titration of dosage.

Elderly patients appear to be more prone to orthostatic hypotension and exhibit an increased sensitivity to the anticholinergic and sedative effects of phenothiazines. In addition, they are more prone to develop extrapyramidal side effects, such as Tardive dyskinesia and Parkinsonism. It has been suggested that elderly patients receive half the usual adult dose. Patients with organic mental disorders or acute confusional states should initially receive one third to one half the usual adult dose, with the dose being increased no more frequently than every 2 or 3 days, preferably at intervals of 7 to 10 days, if possible. After clinical improvement occurs, periodic attempts should be made to discontinue medication.

Dental

The peripheral anticholinergic effects of Phenothiazines may decrease or inhibit salivary flow, especially in middle-aged or elderly patients, thus contributing to the development of caries, periodontal disease, oral candidiasis, and discomfort.

Extrapyramidal reactions induced by Phenothiazines will result in increased motor activity of the head, face, and neck. Occlusal adjustments bite registrations, and treatment for bruxism may be made less reliable.

The Leukopenic and Thrombocytopenic effects of Phenothiazines may result in an increased incidence of microbial infection, delayed healing and gingival bleeding. If Leukopenia or Thrombocytopenia occurs, dental work should be deferred until blood counts have returned to normal, and patients should be instructed in proper oral hygiene, including caution in use of regular toothbrushes, dental floss, and toothpicks.

Surgical

Patients receiving Phenothiazines who must undergo surgery should be closely monitored for hypotension. Also, dosage reductions of anesthetics and other CNS depressants are recommended.

Interactions with Other Medicament

- Adrenaline must not be used in patients who have overdosed with prochlorperazine maleate.
- The CNS depressant actions of neuroleptic agents may be intensified (additively) by alcohol, barbiturates and other sedatives. Respiratory depression may occur.
- Anticholinergic agents may reduce the antipsychotic effects of neuroleptics and mild anticholinergic effect of neuroleptics may be enhanced by other anticholinergic drugs, possibly leading to constipation, heat stroke, etc.
- Some drugs interfere with absorption of neuroleptic agents: antacids, anti-Parkinson drugs and lithium.
- Where treatment for neuroleptic-induced extrapyramidal symptoms is required, anticholinergic antiparkinsonian agents should be used in preference to levodopa, since neuroleptics antagonise the antiparkinsonian action of dopaminergics.
- High doses of neuroleptics reduce the response to hypoglycaemic agents, the dosage of which might have to be raised.
- The hypotensive effect of most antihypertensive drugs especially alphaadrenoceptor blocking agents may be exaggerated by neuroleptics.
- The action of some drugs may be opposed by phenothiazine neuroleptics;these include amfetamine, levodopa, clonidine, guanethidine, adrenaline.
- Increases or decreases in the plasma concentrations of a number of drugs, e.g. propranolol, phenobarbital have been observed but were not of clinical significance.
- Simultaneous administration of desferrioxamine and prochlorperazine hasbeen observed to induce transient metabolic encephalopathy characterised by loss of consciousness for 48 – 72 hours.
- There is an increased risk of arrhythmias when neuroleptics are used withconcomitant QT prolonging drugs (including certain antiarrhythmics, antidepressants and other antipsychotics) and drugs causing electrolyteimbalance.
- There is an increased risk of agranulocytosis when neuroleptics are usedconcurrently with drugs with myelosuppressive potential, such as carbamazepine or certain antibiotics and cytotoxics.
- In patients treated concurrently with neuroleptics and lithium, there have been rare reports of neurotoxicity.

Pregnancy and Lactation

Pregnancy

- Animal studies are insufficient with respect to reproductive toxicity. However, potential harmful effect in animals cannot be ruled out. There is inadequate evidence of safety in pregnancy. Data from epidemiological studies do not suggest a risk of congenital malformations in children exposed in utero to Prochlorperazine.
- As a precautionary measure, Prochlorperazine should be avoided during pregnancy unless the potential benefits outweigh the potential risks.
- Neuroleptics may occasionally prolong labour and at such time should be withheld until the cervix is dilated 3 – 4 cm. possible adverse effects on the neonate include lethargy or paradoxical hyperexcitability, tremor and low apgar score.
- Neonates exposed to antipsychotics (including Prochlorperazine) during the third trimester of pregnancy are at risk of adverse reactions including extrapyramidal and/or withdrawal symptoms that may vary in severity and duration following delivery. There have been reports of agitation, hypertonia, hypotonia, tremor, somnolence, respiratory distress, or feeding disorder. Consequently, newborns should be monitored carefully.

Breast-feeding

Phenothiazines may be excreted in milk, therefore breast feeding should be suspended during treatment.

Side Effects

Generally, adverse reactions occur at a low frequency; the most common reported adverse reactions are nervous system disorders.

Immune system disorders:

- Type I hypersensitivity reactions such as angioedema and urticaria.

Blood and lymphatic system disorders:

- A mild leukopenia occurs in up to 30% of patients on prolonged high dosage.
- Agranulocytosis may occur rarely: it is not dose related
- Hyperprolactinaemia which may result in galactorrhoea, gynaecomastia, amenorrhoea and impotence.

Nervous system disorders:

- Acute dystonia or dyskinesias, including oculogyric crisis, usually transitory are commoner in children and young adults, and usually occur within the first 4 days of treatment or after dosage increases.
- Akathisia characteristically occurs after large initial doses.
- Parkinsonism is more common in adults and the elderly. It usually develops after weeks or months of treatment. One or more of the following may be seen: tremor, rigidity, akinesia or other features of Parkinsonism. Commonly just tremor.
- Tardive dyskinesia: If this occurs it is usually, but not necessarily, after prolonged or high dosage. It can even occur after treatment has been stopped.

Dosage should therefore be kept low whenever possible.

- Insomnia and agitation may occur.
- Convulsions.

Eye disorders:

Ocular changes and the development of metallic greyish-mauve coloration of exposed skin have been noted in some individuals mainly females, who have received chlorpromazine continuously for long periods (four to eight years). This could possibly happen with Prochlorperazine.

Cardiac disorders:

- ECG changes include QT prolongation (as with other neuroleptics), ST depression, U-Wave and T-Wave changes.
- Cardiac arrhythmias, including ventricular arrhythmias and atrial arrhythmias, A-V block, ventricular tachycardia, which may result in ventricular fibrillation or cardiac arrest have been reported during neuroleptic phenothiazine therapy, possibly related to dosage. Pre-existing cardiac disease, old age, hypokalaemia and concurrent tricyclic antidepressants may predispose.
- There have been isolated reports of sudden death, with possible causes of cardiac origin, as well as cases of unexplained sudden death, in patients receiving neuroleptic phenothiazines.

Vascular disorders:

- Hypotension, usually postural, commonly occurs. Elderly or volume depleted subjects are particularly susceptible; it is more likely to occur after intramuscular injection.
- Cases of venous thromboembolism, including cases of pulmonary embolism and cases of deep vein thrombosis have been reported with antipsychotic drugs

– Frequency unknown

Gastrointestinal disorders:

Dry mouth may occur.

Metabolism and nutrition disorders:

- Hyponatraemia
- Syndrome of inappropriate antidiuretic hormone secretion (SIADH).

Respiratory, thoracic and mediastinal disorders:

- Respiratory depression is possible in susceptible patients.
- Nasal stuffiness may occur.

Hepatobiliary disorders:

- Jaundice, usually transient, occurs in a very small percentage of patients taking neuroleptics. A premonitory sign may be sudden onset of fever after one to three weeks of treatment followed by the development of jaundice.
- Neuroleptic jaundice has the biochemical and other characteristics of obstructive jaundice and is associated with obstruction of the canaliculi by bile thrombi; the frequent presence of an accompanying eosinophilia indicates the allergic nature of this phenomenon. Treatment should be withheld on the development of jaundice.

Skin and subcutaneous tissue disorders:

- Contact skin sensitisation may occur rarely in those frequently handling preparations of certain phenothiazines
- Skin rashes of various kinds may also be seen in patients treated with the drug.
- Patients on high dosage should be warned that they may develop photosensitivity in sunny weather and should avoid exposure to direct sunlight.

General disorders and administration site conditions:

- Neuroleptic malignant syndrome (hyperthermia, rigidity, autonomic dysfunction and altered consciousness) may occur with any neuroleptic
- Intolerance to glucose, hyperglycaemia

Pregnancy, puerperium and perinatal conditions:

Drug withdrawal syndrome neonatal

- ADR: stiff neck, torticollis

Symptoms and Treatment of Overdose:

- Symptoms of phenothiazine overdosage include drowsiness or loss of consciousness, hypotension, tachycardia, ECG changes, ventricular arrhythmias and hypothermia. Severe extrapyramidal dyskinesias may occur.
- If the patients is seen sufficiently soon (up to 6 hours) after ingestion of a toxic dose, gastric lavage may be attempted. Pharmacological induction of emesis is unlikely to be of any use. Activated charcoal should be given. There is no specific antidote. Treatment is supportive.
- Generalised vasodilatation may result in circulatory collapse; raising the patient's legs may suffice, in severe cases, volume expansion by iv fluids may be needed; infusion fluids should be warmed before administration in order not to aggravate hypothermia.
Positive inotropic agents such as dopamine may be tried if fluid replacement is insufficient to correct the circulatory collapse. Peripheral vasoconstrictor agents are not generally recommended; avoid the use of adrenaline (epinephrine).
Ventricular or supraventricular tachy-arrhythmias usually respond to restoration of normal body temperature and correction of circulatory or metabolic disturbances. If persistent or life threatening, appropriate antiarrhythmic therapy may be considered. Avoid lidocaine and, as far as possible, long acting anti-arrhythmic drugs.
- Pronounced CNS depression requires airway maintenance or, in extreme circumstances, assisted respiration. Severe dystonic reactions usually respond to procyclidine (5-10mg) or orphenadrine (20-40mg) administered intramuscularly or intravenously. Convulsions should be treated with iv diazepam. Neuroleptic malignant syndrome should be treated with cooling.
Dantrolene sodium may be tried.

Packing

Packed in blister-packs of 10 x 10's / Box.

Pharmaceutical Precautions

Store below 30°C in a dry place, protected from direct light.
Keep away from reach of children.

Expiry Date

3 years from date of manufacturer

Name and Address of Product Registration Holder/ Manufacturer by:

TERAPUTICS SDN. BHD. (590500-W)
Lot 10 & 11, PERDA Industrial Park,
Lorong IKS Simpang Ampat B,
14100 Simpang Ampat, S.P.S.,
Pulau Pinang, Malaysia

Name and Address of Distributor:

ZONTRON PHARMACEUTICALS SDN.BHD. (445695-T)
Lot 10 & 11, PERDA Industrial Park,
Lorong IKS Simpang Ampat B,
14100 Simpang Ampat, S.P.S.,
Pulau Pinang, Malaysia

Malaysian Drug Registration No.

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